

Standard Operating Procedure

Surgical Safety Checklist — Site Marking, Time-Out and Sign-Out

«Hospital Name» — Supports COP.14.d, COP.14.e (CORE Stop-Work)

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» prevents wrong-patient, wrong-site, and wrong-procedure surgery through site marking, a mandatory time-out, and a documented sign-out before the patient leaves the operating theatre.

2. Scope

Applies to every surgical procedure, and to any invasive procedure involving laterality, multiple structures, or multiple levels.

3. Responsibility

Operating Surgeon: marks the site, leads the time-out, and confirms the sign-out.

Entire OT Team: every member present and actively participating in the time-out — this is not one person's job.

Circulating Nurse: documents the time-out and sign-out in the patient record.

4. Procedure

4.1 Site marking

1. Mark the site for every procedure involving laterality (left/right), multiple structures (e.g. fingers, toes), or multiple levels (e.g. spine).
2. Make the mark unambiguous, using a method that remains visible after skin preparation and draping.
3. Involve the patient or relative in confirming the site whenever possible, before the patient is sedated.

4.2 Time-out — before skin incision

1. Pause the entire team immediately before incision. Do not proceed until this step is complete.
2. Confirm aloud, with every team member's active agreement: correct patient identity, correct procedure, and correct surgical site.
3. Any team member — regardless of role or seniority — has the authority to stop the time-out if something doesn't match. Resolve the discrepancy before proceeding.
4. Document the time-out in the patient record, including that all three elements were confirmed and by whom.

4.3 Sign-out — before the patient leaves the OT

1. Before the patient leaves the operating theatre, the team confirms: the procedure performed, instrument/sponge/needle counts are correct, specimens are correctly labelled, and any equipment issues are noted for follow-up.
2. Document the sign-out in the patient record.

5. Stop-Work Authority

Do not begin the incision without a completed, documented time-out confirming patient identity, procedure, and site.

Any team member who identifies a mismatch or an incomplete time-out stops the procedure from proceeding until it is resolved — this includes junior staff stopping a senior surgeon.

Stop-work applies to elective progression of the procedure — it does not block emergency life-saving intervention already underway.

The person who stops documents what was found and how it was resolved. Stopping an incomplete time-out is not a disciplinary matter, for any team member regardless of seniority.

6. Records

Supported by Records 4 and 5 of the COP.14 Required Records pack: Site-Marking Record and Time-Out Record.

7. Related Documents

- HCO_COP_14_v2_REWRITE_DRAFT — COP.14 policy

8. Review

Reviewed annually, and immediately after any wrong-site or wrong-procedure near-miss.

9. References

- World Health Organization. (2008). WHO Surgical Safety Checklist (first edition). <https://iris.who.int/items/08d5cc88-060c-4f03-8a94-330c095d49bc> — the Time-Out and Sign-Out structure in this SOP is adapted from the WHO checklist's three-phase model (Sign In, Time Out, Sign Out).
- Haynes, A. B., et al. (2009). A surgical safety checklist to reduce morbidity and mortality in a global population. New England Journal of Medicine, 360(5), 491-499. <https://doi.org/10.1056/NEJMsa0810119> — the original validation study showing reduced mortality and complications with checklist use.